

Introduction to Behavioral Dentistry



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Outline



- Lecture Learning Outcomes
- Introduction
- Why study behavioral dentistry?
- Definitions
- Determinants of Health
- Behavioral problems
- Health Behaviour
- **BEHAVIOR CHANGE: SOME KEY CONCEPTS**

Lecture Learning Outcomes

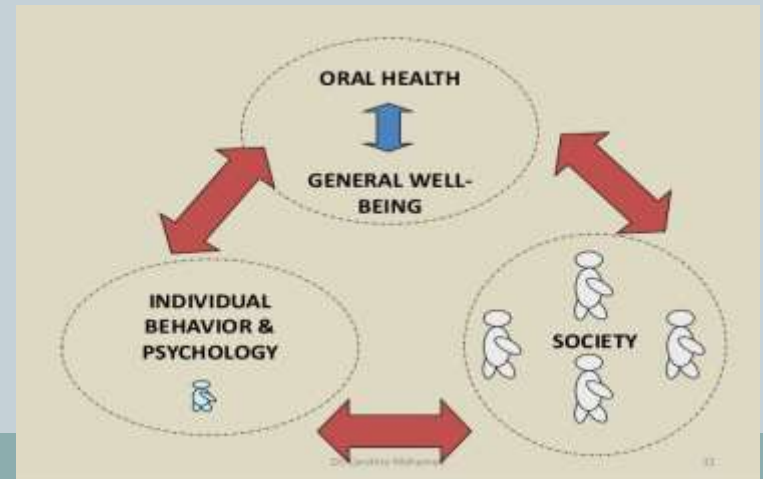


- Name the terminologies related to psychology and behavioral dentistry
- State the key concepts related to challenges of behavior change illustrated from both the patient's and the clinician's perspectives.

Introduction



- There is increasing evidence suggesting oral health status can affect general health and quality of life in people of all ages. Most oral diseases are common chronic diseases.
- Some etiologies of oral diseases are modifiable and associated with the influence of health behaviors as determinants of disease prevalence.



Introduction-Why study behavioral dentistry?



- Populations are becoming increasingly heterogeneous, migrating longer distances, and bringing with them different cultural expectations and needs.
- The cultural heterogeneity impacts on the management of oro-dental diseases, including etiological risk factors (related to harmful lifestyle habits) through behavioral differences displayed by patients from different cultures.
- Training a dental workforce that is culturally and linguistically competent and that values the behavioral and psychosocial needs of multicultural populations is important.
- Dental professionals have long been aware that the person receiving their dental services is likely to confront a variety of emotional and sensory experiences, and to cope with these situations by invoking numerous behaviors.
- A dental workforce that will not only have the potential to reduce oral health inequalities, but also to deliver any communication, training, and clinical management with understanding, respect, and dignity needs to be developed.

Determinants of Health



- Many factors influence the opportunity for patients and clinicians alike to achieve the goal of attaining health and continued wellness.



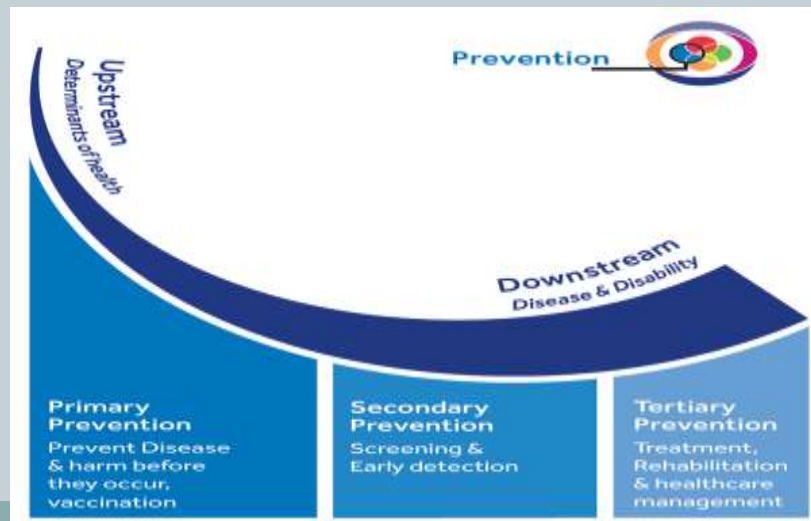


- The acknowledgement of the potential impact of a variety of influences on health status allows the health professional to work with the patient to understand the **individual approach** for optimal wellness.

Why study behavioral dentistry? Conti.....



- Health care challenges are becoming more diverse, with an increasing percentage of the population in the developed world being diagnosed with health decline associated with “lifestyle” behaviors.
- Therefore, the health professional is continuously presented with a dual focus — control of current disease while facilitating the understanding of continuous self - management as part of an effective and equitable long - term solution.





- As a consequence, services for primary and secondary prevention on an individual level oriented toward the change of inappropriate behavior will become a professional responsibility for all oral health care providers.

Definitions



- **Behavioral Science:** The science of the study of human behavior at the level of their own self, other individuals, family and community members, and the resulting reaction on the dental health programs.

Hiremath SS. Textbook of preventive and community dentistry. Elsevier India; 2011 Aug 15.



- **Psychology:** Defined as the study of human behaviour-of how people behave and why they behave in just the way they do.
- **Social psychology:** It is concerned with the psychology of individuals living in human society or groups. It deals with human nature and attitudes in general. Social psychology studies how and why perceptions, thought, opinions, attitudes and behavior vary in different groups and societies (soben).

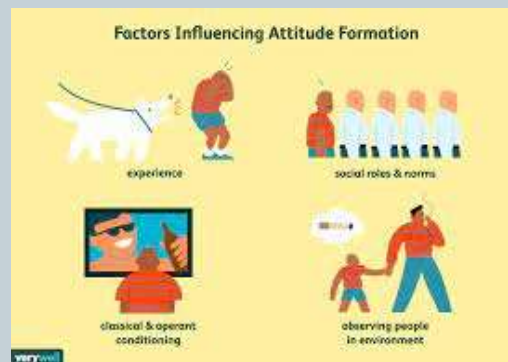


- Social psychology emphasis on understanding the basis for perception, thought, opinion, attitudes, general motivation and learning in individuals and how these vary in human societies and groups.
- It deals with the effect of social environment on persons, their attitudes and motivation.
- Emotions motivate human behaviour, an emotional experience is characterized by both external and internal changes.

Social Attitudes



- As a person grows and develops in life, social attitudes begin to influence behavior and a cultural way of thinking. We tend to internalize the attitudes of others around us by making their attitudes our own. The process through which society influences individuals is called **socialization**.



Behavioral problems from patients' perspective



- Challenges related to poor drug compliance, problems with special needs children, emotional instability, and a lack of awareness of one's disease status or self-image.
- Emotional responses are unique in each patient; for example, those related to pain tolerance and coping strategies for a single clinical decision could influence adherence to medical care and drug compliance.



Chu SY, Lin CW, Lin MJ, Wen CC. Psychosocial issues discovered through reflective group dialogue between medical students. BMC medical education. 2018 Dec 1;18(1):12.

Corah NL, O'shea RM, Skeels DK. Dentists' perceptions of problem behaviors in patients. The Journal of the American Dental Association. 1982 Jun 1;104(6):820-23.

Dentists' Perspective- Patients' Principal Behavioral Problems



- Fee collection; fearful patients; poor motivation for home care and plaque control; ignorance of benefits of preventive care; not following instructions; insurance work (complicated forms, patients' ignorance of benefits); poorly behaved children; and broken, cancelled and late appointments.
- Other problems: Demands for unnecessary extractions; seeking only emergency care; not following instructions concerning orthodontic appliances and postoperative procedures; parents' neglect of children's teeth.

Ingersoll, T.G., and others. A survey of patient and auxiliary problems as they relate to behavioral dentistry curricula. J Dent Educ 42:260-263,1978.

Health Behaviour



- Any activity undertaken by an individual, regardless of actual or perceived health status, for the purpose of promoting, protecting or maintaining health, whether or not such behaviour is objectively effective towards that end.
- However, health professionals working with individual patients, helping people change their behavior is still an important task within their clinical practice.

Reference: Health Promotion Glossary, 1986

Daly B, Batchelor P, Treasure E, Watt R. Essential dental public health. OUP Oxford;

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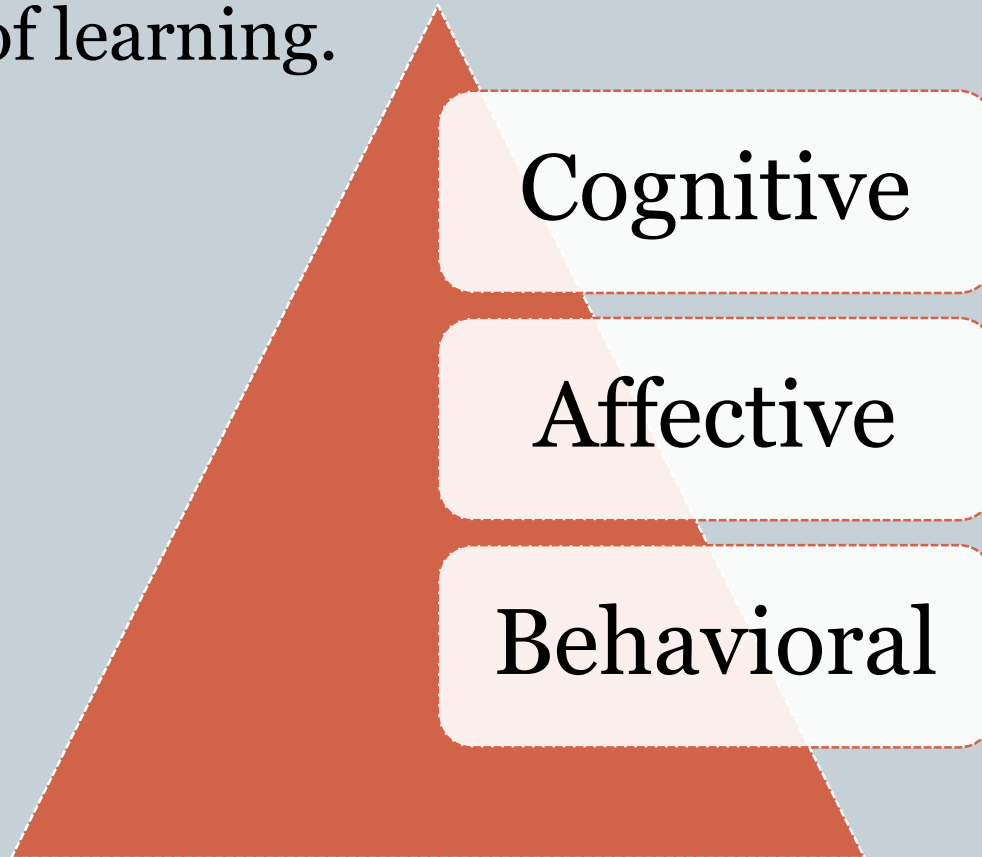


- **BEHAVIOR CHANGE: SOME KEY CONCEPTS**

THREE DOMAINS of LEARNING



- Educational theory has identified that there are three domains of learning.





- The cognitive domain refers to acquisition of factual knowledge and intellectual understanding of ideas.
- The affective domain is concerned with attitudes, beliefs and values.
- Whereas, the behavioural refers to skill or actions performed.

THE OPPORTUNITY IN THE DENTAL SETTING



- Traditionally, dental health education was based upon the theory that acquiring new knowledge would alter attitudes and lead to a change in behavior, the so called KAB model.
- In reality, a very complex and dynamic relationship operates between the three domains of learning.





- Historically, dental clinicians have been characterized as “active,” “powerful,” and “expert,” while patients have been described as “passive” and “cooperative.”
- The dental treatment room itself, where the patients are in a submissive position and the clinicians are in a controlling position, supports these traditional roles.
- With a focus on technical expertise, dental clinicians may believe that their communications with patients will be based on common sense or are secondary to the provision of successful treatment.

- Some dental clinicians struggle with interviewing skills, may miscalculate how much (and how) information should be shared, have difficulty detecting and resolving issues with patient cooperation, or have varying levels of skill in interpreting non - verbal behaviors.





- Consequently, current advice - giving or health education approaches appear to be unpredictable in accomplishing long - term change, potentially leading to frustration of both the patient and the clinician. Yet, the patient may be blamed for poor compliance and further oral health education may be judged as pointless.



Adoption and integration of health behavior change



- Compliance with medical recommendations is generally poor across all chronic disease regimens, which increases health care expenditures and prevents patients from achieving the full benefit of health behavior interventions.



- Health care providers may **harbor an unwarranted sense of their own importance in inspiring behavior change**, ignoring other variables that impact a patient's behavior. This may **serve to diminish the patients' key role** rather than empowering the patients themselves.
- Although health care professionals typically believe that they are providing quality care, the patient perspective of quality could be very different, as there may be a fundamental disagreement of needs and expectations in the clinician - patient relationship.
- Even if there is agreement, the acceptance of care or behavior change (adoption) and the practical application (integration) of care or behavior change often requires **further exploration** to ensure ongoing success.

Importance of communication and collaboration



- Many oral health professionals underestimate the importance of communication as compared to technical skill. This tends to foster a focus on compliance rather than collaboration.
- The traditional model of clinician - patient roles diminishes the value of the communication necessary for successful partnership.
- The nature, frequency, and longevity of clinician - patient interaction within the dental setting are unlike any other health care environment. Therefore, the opportunity to work with patients toward the adoption and integration of positive health behaviors into their current lifestyle is limitless.



Challenges of Behavior Change



- Behavior change requires effort.
- It involves consciously making different choices or adopting new habits and lifestyle patterns and is seldom comfortable, easy, or convenient.



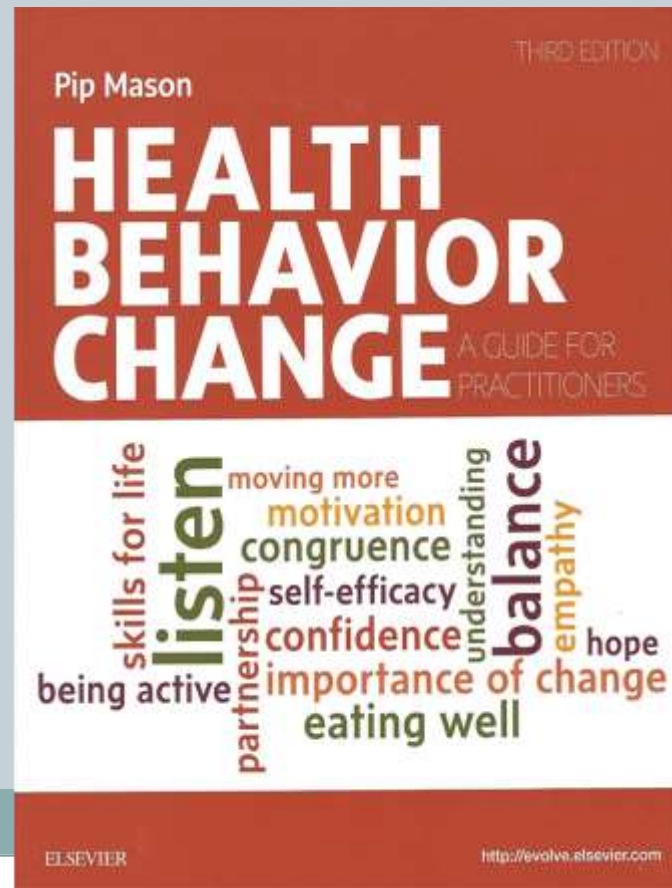
The Factors Determine Behavior Change



- Within the study of health behavior, theories vary in their philosophy of which factors determine behavior change.
- Some place greater emphasis on **individual factors** such as cognitions or emotions, while others include **environmental factors** such as socio - economic status or the influence of the family.
- However, the evidence to date suggests that behavior change is a challenging task and no single approach guarantees success.

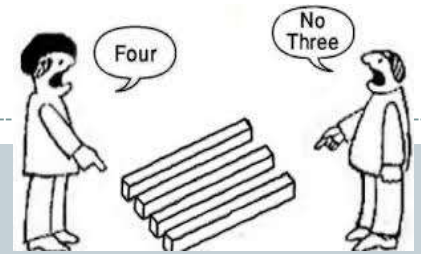


- Behavior change is complex and can seem like a **STRUGGLE** for both the **patient** and the **clinician**.



Working with Behavior change-Concepts, Struggles:

Patient 's Perspective



- 1. Change can happen naturally in everyday life
- 2. Intrinsic motivation affects patient behavior
- 3. People change when they are ready: The concept of readiness
- 4. Ambivalence (feeling two ways about something) is part of the process
- 5. Change happens in the context of our patients ' lives

Clinician ' s Perspective



- As a dental clinician, in any consultation, you may have as little as 5 or 10 minutes to speak with your patient about changes they could be making to improve their oral health.
- There are ways of approaching the challenge of health behavior change that make it less stressful for the clinician and with a greater potential for effecting results in a brief period of time.



- 1. Limitations of giving advice
- 2. The role of ambivalence
- 3. Interpersonal skills (empathy) — a key clinical tool
- 4. Limitations of a “ fix it ” approach
- 5. Agreeing on priorities

Cosmetic dentistry-Concerns with Facial Appearance



- Appearance is considered important by many
- Attractiveness appears to be associated with achieving happiness in life.



- The desire to make changes to appearance could be a symptom of a lack of self-worth, negative self-image, or a psychiatric condition called body dysmorphic disorder (BDD).



Body Dysmorphic Disorder (BDD)



- Some people find themselves so repellant that they dare not leave the house, while the physical abnormality is so minor that concern about it is clearly excessive. Such individuals are likely to suffer from a psychiatric condition.



- Strong tendency to report to dentists with a request for cosmetic dental interventions such as whitening, extensive facade work, and orthodontic treatments.



- It is important to remain vigilant and practice restraint with patients with unusual esthetic requests.

References



- Chapters 1 & 2: Ramseier C, Suvan JE, editors. Health behavior change in the dental practice. John Wiley & Sons; 2011 Jun 9.
- Chapters 8: Mostofsky DI, Fortune F. Behavioral dentistry. John Wiley & Sons; 2013 Dec 31.



Thank You